Case study: Ivy

Condition: **Stroke – right hemisphere ischaemic stroke**

81-year-old woman · Acute stroke unit, public hospital

Ivy Papadopoulos is an 81-year-old woman who was admitted to the acute stroke unit eight days ago with a **right hemisphere ischaemic stroke**. Ivy has been widowed for six years and lives alone in a single-storey house. Her son, Nick, lives in Toowoomba, approximately two hours away, and is listed as her next of kin. Before her stroke, Ivy was independent in all daily activities, attended her local church every Sunday and drove herself to the shops.

Ivy has mild weakness in her left arm and leg. Physiotherapy has documented that she is safe to mobilise independently on the ward with a four-wheeled walker. A brief cognitive screen completed by the medical team two days ago is documented as "within normal range", and the medical team has determined that Ivy is medically fit for discharge home today.

You have been asked to complete a pre-discharge kitchen assessment in the ward's occupational therapy kitchen, in which Ivy will make a cup of tea. Ivy is bright and chatty. She tells you about her friends at church and shows you photos of her five grandchildren on her phone. "I've made thousands of cups of tea, love," she says. "This will be easy."

On the way into the kitchen, Ivy's walker catches the left side of the door frame twice. She does not comment and only gets through after you guide the walker. Ivy explains that at home she boils water in a small saucepan rather than a kettle. She fills a saucepan and places it on the front right hotplate, but turns on the back left hotplate and then walks to the fridge to look for milk. You turn the hotplate off. When you point out that the saucepan is not heating, Ivy says, "Give it a minute, dear, that stove is always slow."

You set out a tray with two cups, teabags, a sugar bowl and teaspoons. Ivy uses the items on the right side of the tray but asks you where the sugar is, although the sugar bowl is on the left side of the tray. She leaves the left-hand cup empty. To talk about her plans after discharge, you show her a short shopping list written in two columns. Ivy reads the four items in the right-hand column and says, "That's all I need."

When you ask how her left arm and leg are going, Ivy says, "There's nothing wrong with that side, dear. The doctors fuss too much." While drinking her tea, she tells you her fridge will be empty when she gets home and she will "just drive down to the shops in the morning", as she always does. Her car is parked in her garage at home.

There is no clear documentation regarding whether Ivy has been given advice about driving since her stroke, whether a home visit has been considered, or what supports, if any, have been arranged for after discharge. It is also unclear whether the medical team noticed any of these difficulties during the cognitive screen.

As you return to the ward, a nurse hands Ivy the ward phone. It is Nick, who has booked a taxi to take her home at 1 pm and wants it confirmed before he starts his shift, as he will not be contactable until tonight. The nurse in charge then tells you that the emergency department has a patient waiting for Ivy's bed and asks whether you have finished. Ivy smiles and says, "See, it's all organised. I'll be home by lunchtime."

You are the occupational therapy student completing Ivy's pre-discharge kitchen assessment and your clinical supervisor is not currently in the room.

Present Ivy's case to your clinical supervisor using the ISBAR framework. Identify the immediate concerns and explain what you would prioritise in responding to the situation.

ISBAR planning sheet

Use your 10 minutes of preparation to plan. Then speak for 7 minutes to your “supervisor”.

I Introduction

≈ 20 s

Who you are, your role, who the client is and why you need to talk now.

S Situation

≈ 40 s

A brief but complete summary of what is happening and the immediate concern.

B Background

≈ 1 min

Relevant history: the primary problem, medical and social history, medications/allergies, interventions so far and OT to date. Name missing information rather than assuming it.

A Assessment

≈ 2 min 20 s

Your interpretation. Case finding → what it may mean → consequence for safety and occupations → priority. Separate facts, interpretation, gaps and proposed assessment. What needs escalation?

R Recommendation

≈ 2 min 40 s

What should happen next, in priority order. Action → reason → who → review. Immediate safety, escalation, MDT input, one short-term and one longer-term OT assessment, intervention, referrals and follow-up.

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